

Hannah's PCP Appointment — Reference Sheet

Prepared June 25, 2026

Today's Priority List

1. **Birth control prescription** — labs show suppressed LH, FSH, and estradiol → HPA axis dysregulation driving hormonal imbalance; ask for hormonal support and a plan
 2. **Valacyclovir Rx** — 1g three times daily × 3-6 months for EBV reactivation (see EBV section below)
 3. **Repeat EBV Early Antigen IgM draw** — need to track trajectory
 4. **Labs: iron, B12, vitamin D** — deficiencies common in Long COVID; can amplify fatigue
 5. **Metabolic panel / LFTs** — kidney and liver check given ongoing medication load
 6. **Weight changes** — gained weight; elevated cortisol + suppressed estradiol on labs may be driving this hormonally; any management plan must be PEM/pacing-compatible — standard exercise advice does not apply
 7. **Anti-cardiolipin IgM follow-up** — positive on prior labs; ask if hypercoagulability workup or rheumatology referral is warranted
 8. **Gut** — ongoing occult blood + Sep 2025 colonoscopy (patchy healing ulcers); starting SIM01 probiotic (B. adolescentis + bifidum + longum)
 9. **No GET on record** — pacing protocol in place; graded exercise therapy is contraindicated per WHO, CDC, and NICE guidelines
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EBV Reactivation & Valacyclovir — Clinical Rationale

Why EBV matters in Long COVID

Dr. Akiko Iwasaki's lab at Yale (published in *Nature* and via the AMA) identified EBV reactivation as a key driver in a subset of Long COVID patients. In their cross-sectional study of 215 participants, elevated EBV antibody responses were one of the distinguishing immune features of Long COVID vs. recovered patients.

Mechanism: acute COVID-19 disrupts immune homeostasis → latent EBV reactivates → ongoing inflammation, fatigue, episodic pain.

My specific evidence

- **June 2025 labs:** EBV Early Antigen (EA) IgM — *reactive* (active reactivation, not just prior exposure)
- **September 2025 colonoscopy:** patchy healing ulcers consistent with EBV-driven mucosal injury
- **Symptoms:** fatigue, episodic body pain — consistent with EBV reactivation profile

Why valacyclovir (not acyclovir)

Valacyclovir is the prodrug form of acyclovir with $\sim 3\times$ better oral bioavailability. Fewer pills, more effective systemic levels.

Requested prescription

| **Valacyclovir 1g TID (three times daily) $\times$ 3-6 months**

This dosing follows the Iwasaki Lab / Yale approach for EBV reactivation in Long COVID. Clinical trials of antivirals targeting EBV in LC are actively being developed (PolyBio Research Foundation, 2024).

References for your doctor

- Iwasaki Lab, Yale School of Medicine:
<https://medicine.yale.edu/lab/iwasaki/projects/immunology-long-covid/>
 - AMA interview with Dr. Iwasaki on EBV + Long COVID:
<https://www.ama-assn.org/public-health/infectious-diseases/latest-long-covid-research-symptoms-testing-and-treatments-akiko>
 - PolyBio EBV antiviral trial (Iwasaki co-investigator):
<https://polybio.org/projects/a-clinical-trial-of-repurposed-hiv-antivirals-in-longcovid/>
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My Phenotype (for context)

Based on 4 lab draws, June–September 2025

- **Gut/Viral persistence** — stool occult blood, colonoscopy findings, EBV EA IgM reactive
 - **EBV reactivation** — confirmed serologically
 - **Hypothalamic-Pituitary dysregulation** — suppressed LH, FSH, estradiol; elevated cortisol
 - **PEM / Dysautonomia** — post-exertional malaise, low sodium and CO₂ on labs
 - **Autoimmune marker** — anti-cardiolipin IgM positive
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Pacing Protocol Note

I am on a structured pacing protocol (Repair Spectrum Framework) based on biometric data (WHOOP + Visible). **Graded Exercise Therapy (GET) is contraindicated** per WHO, CDC, and NICE for ME/CFS and Long COVID with PEM. Please document this in my chart.